

Burns –Children (Age 0 – 16 yrs)

Applies to:

Scalds (Hot liquids, steam, grease)

Fire

Contact burn (hot object)

Chemical

Electrical

Burns above 5% should have the major burns protocol activated.

Patient Details

Full Name:

DOB:

Unit no:

Seen By:

Date:

Time:

Child's weight (kg)

CAUSE FOR CONCERN

If history / examination reveals any

of the following:

Trapped in a confined space

Carbonaceous sputum

Stridor, hoarse voice, Dysphagia

Severe / multiple trauma

High voltage electrical burn.



1. Inform senior ED ST 4 + immediately
2. Contact resus
3. Contact Paediatric anaesthetist if potential / any airway compromise

Chemical Burn.

If powder chemical ensure all powder removed before irrigation

If corrosive metal ensure any particles are removed with tweezers

Most chemicals should be irrigated for a minimum of 20 mins with water

Always refer to Toxbase online or national poisons information service.

pH on arrival

pH after 20 minutes of irrigation

Continue to irrigate and document PH every 20 mins until PH 7.

Mechanism

☐

scald

☐

Fire

☐

Contact

☐

Other

☐

Chemical → Substance =

☐

Electrical → Needs ECG.

Account of burn:

Time of burn:

First aid given:

Who was supervising the child?

Depth of burn (use charts on pages 2 and 3):

Percentage of burn (use charts on pages 2 and 3):

PMH:

Allergies

Drug Hx:

immunisations:

☐

All children under the age of 5yrs must have an A-form completed

Pain score

0 1 2 3

4 5 6

7 8 9 10

Pain score:

Time:

Mild to moderate pain (score 1-6)

Give paracetamol

And or ibuprofen

Severe pain (scores 7-10).

As for mild to moderate pain, Plus Entonox (as a holding measure)

Then:

Opiate analgesia

Consider the need for amfetop.

Safeguarding concerns?

Yes

No

Is there a consistent history?

Does the injury match the description of events?

Is the injury appropriate for the Child's developmental stage?

Any delay in presentation has a Satisfactory explanation?

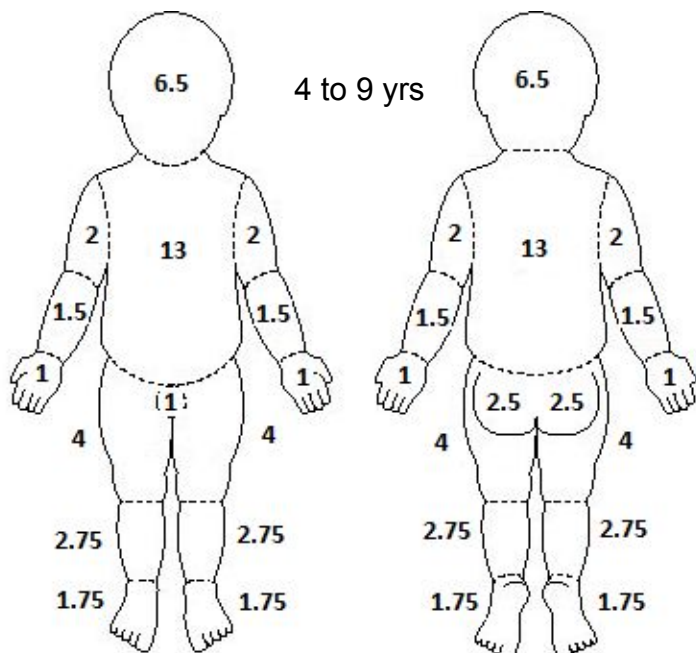
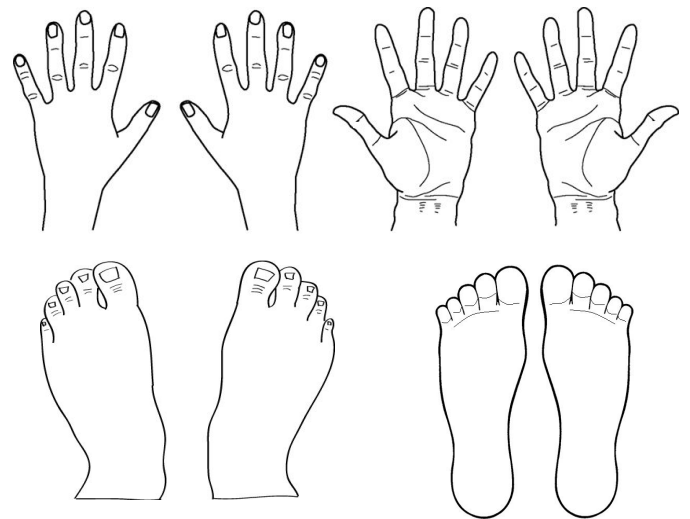
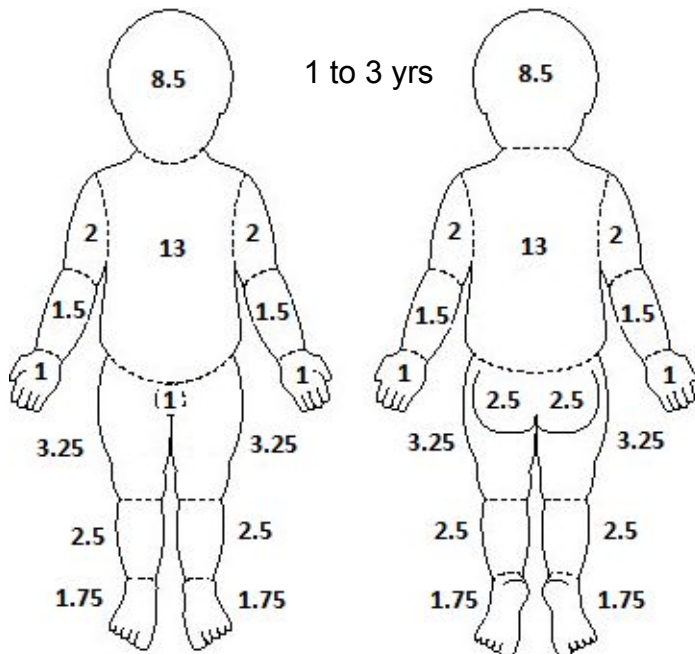
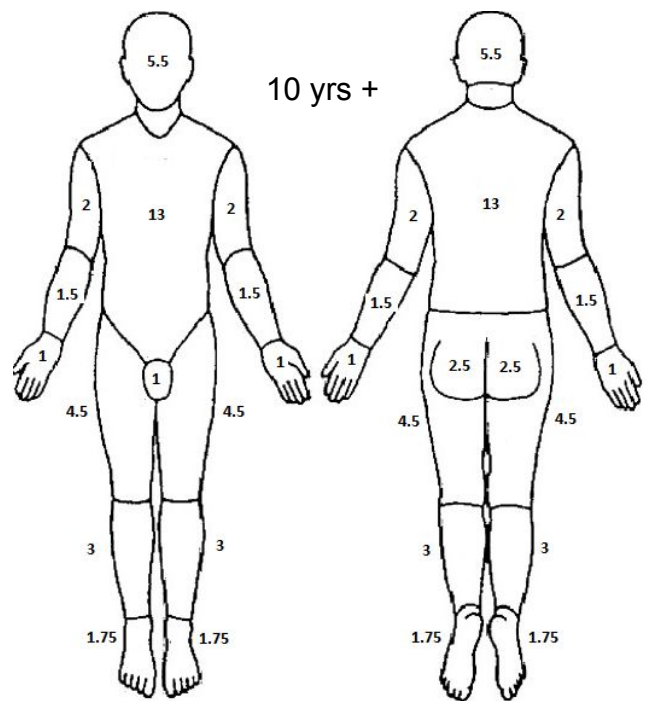
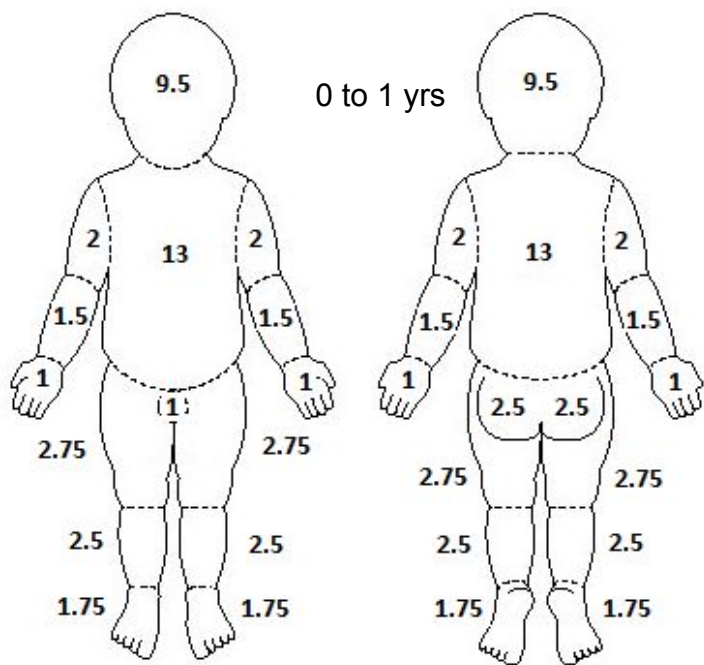
No other injury / unexplained finding On examination?

Are the parent and child interacting / Behaving appropriately?

If you answered no to any of the Above, Discuss the patient with ST4+.

Notes continuation

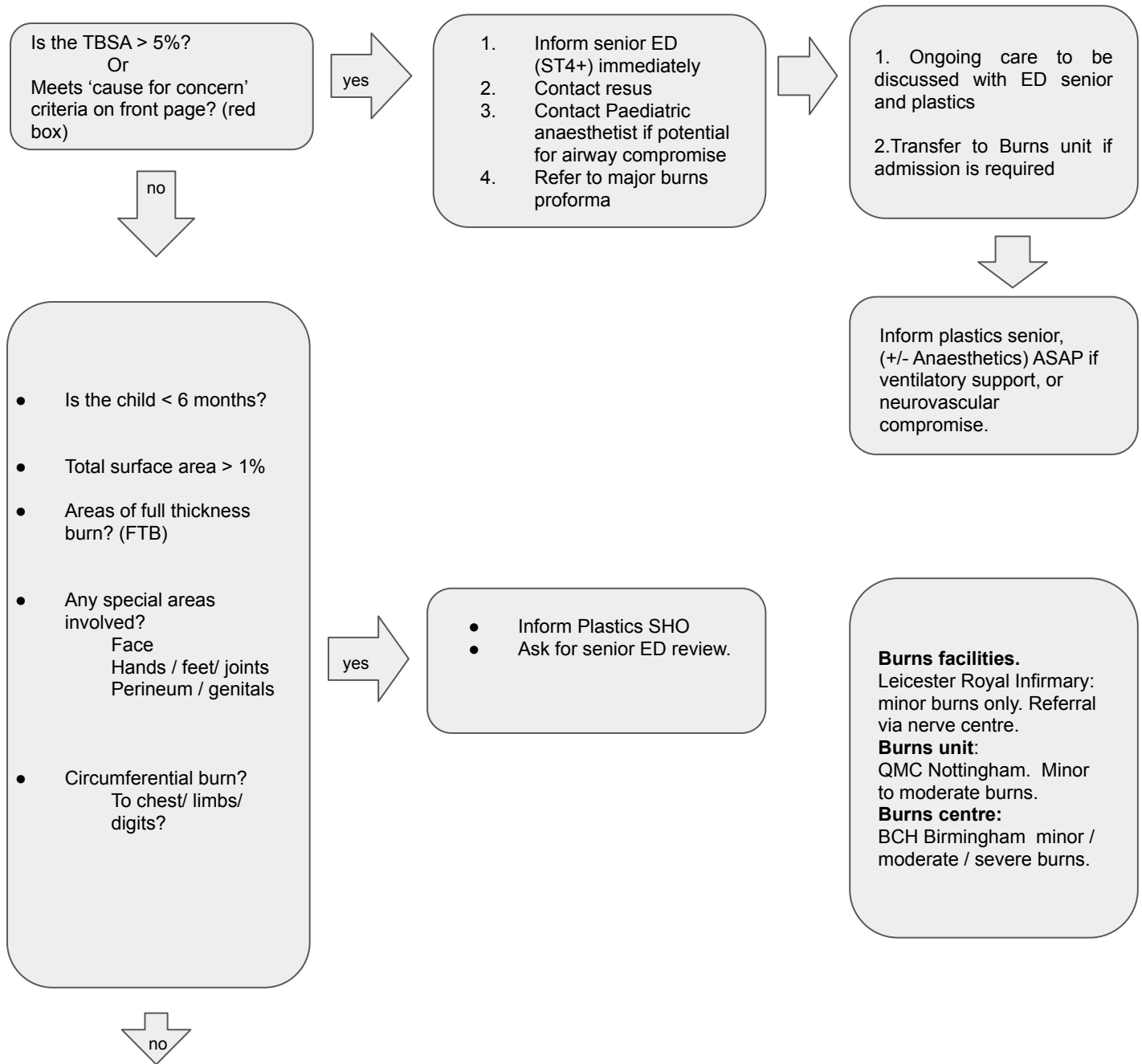
	Epidermal (superficial)	Partial thickness Superficial Dermal	Partial thickness Deep Dermal	Full thickness
Appearance	Red shiny	Pale pink, mottled, blistered	Cherry red, blistered	Dry, leathery, white/ black/ charred. No blisters
Blanching to pressure	Yes - brisk	Yes - brisk	No	No
Sensation	Painful	Painful	Dull	Absent



Region	% partial thickness	% full thickness
Head		
Neck		
Anterior trunk		
Posterior trunk		
Right arm		
Left arm		
Buttocks		
Genitalia		
Right leg		
Left leg		
Total		
TBSA		

Management

Management depends on Age, depth, total body surface area, site and type of burn.



Wound cleansing and dressing

After initial 1st aid, cover all burns with cling film to aid pain relief.

Cleansing.

1. All wounds and surrounding skin are thoroughly cleaned and remove any loose or dead skin.
2. De-roof any large blisters

Dressings.

1. Dress all burns with an antimicrobial dressing (such as Urgotul Ag.)
2. Ensure adequate padding is used (all burns lose lots of fluid)

Follow up advice.

All burns dressings need to be reviewed 48 hrs after injury, either by paed community nurses, or BPDC (burns and plastics dressing clinic).

1. If the burn is 1% or less, or superficial dermal, refer to community nurses for re-dressing and further management.
2. Refer to BPDC via the plastics SHO if >1%
3. Give parent or guardian burns information leaflet.
4. Give verbal advice -: regular analgesia
: red flag advice regarding toxic shock symptoms.

Do not prescribe oral antibiotics in acute burns.